

=====

FACSIMILE TRANSMISSION

TO: Prior Authorization Review

RE: Clinical note - Skilled nursing facility admission - diabetes stabilization and rehabilitation

DATE: 2021-10-15

PAGES: 1

=====

****Subjective:**** Traci Wiegand is a 65-year-old woman admitted for skilled nursing facility care for stabilization of type 2 diabetes mellitus and functional rehabilitation. Chief complaints: blurred vision and tingling in the hands and feet. She describes several months of poor glycemic control with documented hyperglycemia and classic symptoms - persistent thirst, fatigue, and mental fog. Vision blurs intermittently, worse in the afternoons, to the point that reading medication labels became difficult; she had a dilated eye examination two weeks ago (tropicamide drops) with outpatient follow-up pending. Tingling of the hands and feet is present a few months, predominantly nocturnal, without burns, cuts, non-healing sores, or falls; she checks her feet inconsistently. Her only medication is 24-hr metformin hydrochloride 500 mg extended-release daily, but adherence and meal timing collapsed over a stressful autumn - erratic meals (crackers for dinner, then prolonged fasting) with correspondingly erratic dosing. Psychosocial context is central: she lives alone, finances tightened after leaving work, her sister and closest confidante died two years ago, and she describes marked social isolation and being overwhelmed by self-management, which drove the decision for skilled-level support. ROS otherwise negative: no fever, chest pain, dyspnea, abdominal pain, dysphagia, focal weakness, or open skin lesions. History includes prediabetes progressing to type 2 diabetes with hyperglycemia, hypertriglyceridemia, metabolic syndrome, anemia, obesity, chronic sinusitis, and stress; medication review was due on arrival. Never-smoker. No known allergies.

****Objective:**** Alert, oriented, pleasant woman, tearful at moments when discussing the admission but engaged and forthcoming; anxious but cooperative. Obese habitus. No acute distress. History and physical examination completed on admission: heart regular; lungs clear; abdomen soft and nontender. Skin intact, including both feet - no ulcers, calluses of concern, or breakdown on initial inspection. Light-touch sensation grossly intact despite reported paresthesias. Gait slow and effortful with reduced endurance, requiring rest with prolonged distances; transfers independent. Reports intermittent blurred vision; eyes without redness or pain. No numeric vital signs or laboratory values recorded in the admission dataset.

****Assessment and Plan:****

Type 2 diabetes mellitus with hyperglycemia

Months of poor control driven by disrupted routines and psychosocial stress rather than medication failure; admitted for skilled stabilization.

- Continue 24-hr metformin 500 mg ER daily, administered consistently with meals; no regimen change at admission.
- Daily nursing care and supplementary surveillance: pre-meal glucose checks, medication administration support, symptom monitoring.
- Consistent-carbohydrate meal plan; dietitian involvement through professional/ancillary services, including a home-translatable meal plan.

Blurred vision

Intermittent blurring in the setting of hyperglycemia; recent dilated examination with tropicamide.

- Track symptoms as glycemic control steadies; maintain scheduled outpatient eye follow-up; report acute visual change immediately.

Tingling in hands and feet

Nocturnal-predominant paresthesias of a few months; skin intact and sensation grossly intact.

ously preserved on admission.

- Daily foot inspection by nursing; protective footwear; report any skin break; reassess symptoms as control improves.

Functional status and rehabilitation

Deconditioned with reduced endurance after a prolonged period of poor health and self-neglect.

- Occupational therapy several sessions weekly: medication organization, kitchen and self-management routines, energy pacing.
- Physical therapy: strength and balance sessions.
- Speech and language therapy: baseline swallow screen and cognitive-communication strategies (checklists, memory aids) supporting regimen self-management.

Hypertriglyceridemia and metabolic syndrome

- Addressed through the consistent-carbohydrate, heart-sensible facility diet and dietitian counseling alongside the diabetes plan.

Stress and social isolation

Bereavement, financial strain, and isolation directly precipitated the loss of self-care.

- Social services engagement during the stay; structured group activities encouraged; individualized plan of care developed with the patient.

Discharge planning

- Goal of a several-week stay; pre-discharge assessment of medication management, meal preparation, mobility, and home supports before discharge home; anticipated return to independent living with a sustainable routine.